



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : صبريه محمد مرعى فرحان

العنوان :-

الحى / المدينة : الصف

المحافظة : الجيزة

تاريخ تسجيل الإستمارة : 07-10-2026

رقم البطاقة : 26003052101961 رقم التليفون : 01501738131

العمر (تاريخ الميلاد) : 05-03-1960 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي : _____
التوقيع: (_____)

المقر بما فيه : _____
التوقيع: (_____)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام رنين مغناطيسي علي الفقرات الصدرية بالصبغة

[12771]

قسم العلاج :
الأورام

التعليق على التشخيص : _____

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان

الوظيفة : _____
الوظيفة : _____
الوظيفة : _____

التوقيع : _____
التوقيع : _____
التوقيع : _____

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة الأبحاث بالمرفقة رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. _____ /د. _____ /د. _____ /د. _____

قرار اللجنة

التشخيص النهائي : _____
خطة العلاج : _____
المستشفى : _____
جهة العلاج : _____
ملاحظات : _____

قسم : _____
نفقات : _____

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: صبريه محمد مرعي فرحان
National ID: 26003052101961



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :
--:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي MRI thoracic spine with contrast

الوقت :

--:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
 وزارة الداخلية
 مصلحة التعداد السكاني

صبريه
 محمد مرعي قرحان
 قرية النيسى
 مركز الصف - الجيزة

٢٦٠٠٣٠٥٢١٠١٩٦١

LN8785837

٢٠٢٥/١٢ ٢٦٠٠٣٠٥٢١٠١٩٦١
 بدون عمل
 أنثى
 رتبة
 مستقلة

البطاقة سارية حتى ٢٠٣٦/١٢/٢٧



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية



04/27/2026

MDT FORM

Name:	Sabria Mohamed Maree Farhan	MRN:	5051
Age:	66 Y	ID:	26003052101961
Menopausal Sta		Phone:	01501738131
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	DM	Reason Of MDT:	POST OP

Clinical Data:

BX: IDC
ER: 8/8 PR: 3/8 HER2: +3 KI67:

Sonomammography:Mets Workup:History:

POST NACT

Recommendations:

POST LT SM +SLNB T1A N0(0/7) PATHO GOOD RESPONSE

MDT Outcome & Plan

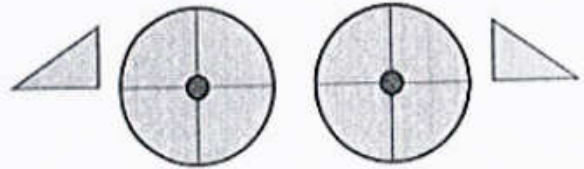
Continue Phesgo

PORT

AI for 5 yrs

R

L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. نوره م. مصطفى	د. لاني حسن				

Name: صبريه محمد مرعي فرحان		
MR. : 5051		
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض	
بقسم:	رقم طبي:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient is a 66-year-old individual who is postmenopausal. The clinical history documents a postmenopausal left breast cancer classified as ct5b, no metastasis, invasive ductal carcinoma grade 3, with estrogen receptor 8/8, progesterone receptor 3/8, HER2 3+, and Ki-67 25 %. The patient received neoadjuvant chemotherapy (tchp) on 26/3/2026, followed by left modified radical mastectomy (ypt1a ypn0/7) and adjuvant endocrine therapy; anti-HER2 therapy was not administered from 3/2026. Biomarker assessment confirms ER-positive, PR-low, HER2-positive disease with a Ki-67 proliferation index of 25 %. Imaging and metastatic workup details are not provided in the form. The MDT outcome and plan specify that the patient will undergo cTS on 31/8/2026 followed by a FREE protocol. The patient received PORT and, given residual disease after surgery, will receive KADCYLA. Ongoing endocrine therapy with AI FEMARA is planned for 7 years, and ZOMETA is to be administered for 6 months.		
تحريراً في ,		
التاريخ :	الوقت :	23-09-2026
الطبيب المعالج :		
يعتمد ,		



Patient Name: Sabriah Mohamed Marei Farhan	Date: 12/03/2025
Exam: C.T ABDOMEN & PELVIS	Age: 65 Year(s)
Medical ID: 795/2025	Pat. ID: 97413

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- Reported case of left breast cancer.

I Chest:

- Left breast upper domain / retroareolar asymmetrical density measuring 3.5 X 5.6 cm.
- Enlarged left axillary level I and II lymph nodes reaching 1 X 1.3 cm.
- Enlarged left internal mammary lymph nodes reaching 1 cm.
- Bilateral pulmonary nodules the largest is right lower lobar medial segment measuring 7 mm.
- Prominent subcentimetric mediastinal lymph nodes.
- No pleural or pericardial collection, pleural thickening or masses.
- cardiomegaly.

II Abdomen & pelvis:

- The liver is cirrhotic with heterogenous attenuation. No sizable focal lesions on non-contrast basis.
- Dilated CBD with intraluminal dense stone measuring 8 mm.
- Normal non contrast CT appearance of the mildly enlarged spleen, pancreas, kidneys (apart from dense left renal upper calyceal stone) and suprarenals glands.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- Heterogenous osseous texture with few vertebral tiny lucent foci..
- Diffuse omental fat smudging and stranding.

Opinion:

- Reported case of left breast cancer revealed:
- Bilateral pulmonary nodules of indeterminate nature for follow up.
- Cirrhotic liver showing no definite hepatic deposits on non-contrast basis.
- CBD stone with biliary dilatation.
- Heterogenous osseous texture for bone scan.

Dr. Mohamed Mohsen, M.D.



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Ministry of Health and Population

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وزارة الصحة والسكان



A-Patient ID:

1- Name : صبرية محمد مرعي فرحان

Sex:

MDT form

3- Medical number:

Female

83265185

Old Med :

Old Med :

795-2025

2-ID number: 26003052101961

Patient Age : 65 Y



Family history

Family History

Phone

Phone

01501738131

BMI

BMI

marital status

Marital Status
MARRIED

NRS Score

NRS Score

GROUP

GROUP

Co morbidity

Co Morbidity

HCV, liver cirrhosis

Menopausal status

Menopausal Status
POSTMENOPAUSAL

Initial STAGE

Initial STAGE

REASON OF MDT

REASON OF MDT
NEW

Date of MDT

Date of MDT

2025-05-03

Breast Imaging :

It breast at 2 oclock with irregular speculated mass with dilated duct reaching nipple 0.3 cm from skin 4.7x2.3 cm

Clinical data

Mets Workup :

non contrast ct cap: bilateral pulmonary nodule (indeterminate)
cirrhotic liver HCV
CBD stone
bone scan:free

BX

IDC

BY COMMENT:

giff with focal DCIS

ER

8/8

ER COMMENT:

PR

3/8

PR COMMENT:

Her 2 Neu

3

Her 2 Neu COMMENT:

SISH

SISH COMMENT:

Ki67

25

T

4B

N

0

M

0

Sonomammography

PATIENT HISTORY

MDT OUTCOME & PLAN

NEW LABS REQUESTED

FOR NEOADJ TCHP X 6 CYCLES THEN RE-ASSES

FOLLOWED BY SM + SLNB

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

SIGNATURE

SURGERY

SURGERY

RADIOLOGY

RADIOLOGY

Psychiatric

Psychiatric

PATHOLOGY

PATHOLOGY

Nutrition

Nutrition

MDT Form

[[DOCTYPE:MDT_FORM]]



Patient Name : الأستاذ / صبريه محمد مرعي فرحان
Patient No. : 26003052101961
Path No. : 525/25
Sex : Female
Age : 64 Y
Referring Doc :

Request No. : 13000215
Request Date : 06/02/2025
Verification Date : 26/02/2025
Financial Category : أورام دار السلام - مياثره
Department : Out Patient
Registered by : Mrs. Aya Mohamed

Pathology Report

Clinical Data: Left breast upper outer quadrant malignant looking mass.
Nature of Specimen: Tru-cut biopsy, collected at 06/02/2025.

Gross Examination:

- Three grayish white tissue cores up to 2 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+3+2) Grade III.
- * Ductal carcinoma in situ: Focal.
- * Perineural invasion: Not detected.
- * Angioinvasion: Not detected.
- * Microcalcifications: Not detected.
- * IPT (Positive control showed appropriate reactivity):
 - ER: Strong nuclear reaction in 70% of tumor cells (score 8/8).
 - PR: Weak nuclear reaction in 5% of tumor cells (score 3/8).
 - Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
 - Ki67: 25% nuclear reaction of tumor cells.

Diagnosis:

Left breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade III, with focal DCIS.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 3/8).

Verified by :

Dr. Toka Medhat

Printing Date : 11/03/2025 09:30:08

Page 1 of 2

Printing By : marwa mohamed youss



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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / صبريه محمد مرعي فرحان
Patient No. : 26003052101961
Path No. : 525/25
Sex : Female
Age : 64 Y
Referring Doc :

Request No. : 13000215
Request Date : 06/02/2025
Verification Date : 26/02/2025
Financial Category : أورام نازي السلام - متاخره
Department : Out Patient
Registered by : Mrs.Aya Mohamed

Pathology Report

- Her2: Positive (score 3+).
- Ki67: Positive in 25% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+ based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Walaa Ghonaim
Dr. Yasmine Essam

Verified by :
Dr.Toka Medhat

Printing Date : 11/03/2025 09:30:08

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Patient Name: Sabriah Mohamed Marei Farhan	Date : 21/12/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 66 Year(s)
Medical ID: 795/2025	Pat. ID: 97413

BILATERAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

Indication: Follow up after neoadjuvant chemotherapy of pathologically proved left breast carcinoma.

- **Breast density:** Scattered densities of glandular tissue . *Breast density matches ACR B.*
- **Evidence of bilateral vascular calcifications .**
- No detected suspicious mass density, architectural distortion or pathological micro-calcification.
- Normal skin and nipple.

By complementary breast ultrasound:

- **Complete disappearance of the previously described malignant mass seen at the UOQ of the left breast .**
- **No other suspicious masses in other quadrants.**
- **No detected suspicious masses in the right breast .**
- Normal skin, subcutaneous fat and nipple areola complex on both sides.
- **No detected significant axillary lymph nodes.**

Conclusion:

- Comparing to the previous radiological datafindings are suggestive of complete radiological response (CR)(BIRARDs 6 category).
- Normal study of the right breast (BIRADS 1 category).

Much Obliged

DR.MONA ZAKY.MD

professor-Breast imaging-Mansoura university

Signature Dr. Mona Zaki

21/12/2025 11:36 AM



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Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



A-Patient ID:

1- Name : صبرية محمد عري فرحان

2-ID number: 26003052101961

MDT form

3- Medical number:
Female

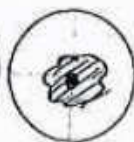
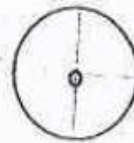
83265185

Old Med :

Old Med :

795-2025

Patient Age : 65 Y



Family history

Family History

Phone

Phone

01501738131

BMI

BMI

marital status

Marital Status
WALA (2)

NRS Score

NRS Score

GROUP

GROUP
B

Co morbidity

Co Morbidity

HCV, liver cirrhosis
dm
open cholelectecto

Menopausal
status

Menopausal Status
POSTMENOPAUSAL

Initial STAGE

Initial STAGE

REASON OF MDT

REASON OF NOT
NEW

Date of MDT

Date of MDT

2025-05-03 ☐

Breast Imaging :

lt breast at 2 oclock with irregular speculated mass with dilated duct
reaching nipple 0.3 cm from skin 4.7x2.3 cm

Clinical data

Mets Workup :

non contrast ct cap: bilatreal pulmonary nodule (indtermiante
cirrhotic liver HCV
CBD stone
bone scan:free

BX
IDC

ER
8/8

PR
3/8

Her 2 Neu
3

SISH

KI67
25

BX COMMENT:
giii with focal DCIS

ER COMMENT:

PR COMMENT:

Her 2 Neu COMMENT:

SISH COMMENT:

T
4B

N
0

M
0

Sonomammography

PATIENT HISTORY

MDT OUTCOME & PLAN

NEW LABS REQUESTED
FOR NEOADJ TCHP X 6 CYCLES THEN RE-ASSES
FOLLOWED BY SM + SLNB

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY
H. Koley

RADIOLOGY

RADIOLOGY

SURGERY

SURGERY
J. S. 25

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition

ECHO REPORT

NAME: *د. محمد مصطفى*

DATE: 21 / 10 / 2025

ID NUMBER: ٧٩٥ / ٢٥

SEX: ♀ AGE: 65

EF	69%	55-75%	PWT	0.9	0.6-1.1cm
LVEDD	4.5	3.5-5.8cm	AO.R	2.3	3-3.7cm
LVESD		2-4cm	LA	3.5	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	1.0	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions *normal*
 LV Wall Thickness *normal*
 LV Diastolic Function *DD grade I*
 SWMA *normal*
 LA and Ao Root Dimensions *normal*
 Mitral Valve *normal*
 Aortic Valve *normal*
 Tricuspid Valve *TR*
 Pulmonary Valve *normal*
 Right Side and Systolic Function *normal*
 Intracardiac Masses and Effusion *normal*

DR.
[Signature]

Patient Name : الأستاذ / صبريه محمد مرعى فرحان	Request No. : 13005581
Patient No. : 26003052101961	Request Date : 26/03/2026
Path No. : 801/26	Verification Date : 08/04/2026
Sex : Female	Financial Category : جوستاف روسي - مبادره
Age : 66 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, Post-NACT.

Nature of Specimen: Left simple mastectomy & Sentinel lymph nodes biopsy.

Gross Examination:

- * Left simple mastectomy specimen measured 16x10x4 cm, with covering skin ellipse measured 16x8 cm. Nipple & areola looked unremarkable grossly.
- * Serialling showed a tumor bed site 5x5x3 cm, located 1 cm from the least deep margin.
- * Sentinel lymph nodes: Fibrofatty tissue piece measured 4x3x1 cm, entangling 7 lymph nodes, the largest measured 1.5x1x0.5 cm.
- Gross was done by; Dr. Heba Hassan.

Microscopic Examination:

- * Histological type: Residual viable invasive duct carcinoma with mucinous differentiation.
- * Grade 2 (Differentiation 2, Pleomorphism 2, Mitosis 2).
- Cellularity (4%), Actual tumor size (0.2x0.2 cm).
- * Ductal carcinoma in situ: 1% High grade ductal carcinoma in situ (Solid pattern).
- * Angioinvasion: Absent.
- * Nipple & areola: Free.
- * Deep margin: Free.
- * Sentinel lymph nodes: Seven lymph nodes free of metastatic deposits (0/7).
- * Histologic response to therapy: Pathological good response to therapy.

Diagnosis:

- Left breast cancer (Post-NACT), Left simple mastectomy & Sentinel lymph nodes biopsy:
- Residual viable invasive duct carcinoma with mucinous differentiation, Grade 2 (Cellularity 4%) (Actual tumor size 0.2x0.2 cm).
 - Ductal carcinoma in situ: 1% High grade ductal carcinoma in situ (Solid pattern).
 - Nipple & areola: Free.

Verified by :

Dr.Duaa Mohamed

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Page 1 of 2

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Patient Name : الأستاذة / صبريه محمد مرعى فرحان
Patient No. : 26003052101961
Path No. : 801/26
Sex : Female
Age : 66 Y
Referring Doc :

Request No. : 13005581
Request Date : 26/03/2026
Verification Date : 08/04/2026
Financial Category : جوستاف روسي - مياره
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

- Deep margin: Free.
- Sentinel lymph nodes: Seven lymph nodes free of metastatic deposits, LNs (0/7).
- Stage: ypT1a, ypN0 (sln).
- Residual cancer burden: 1.572, RCB-II.

Signature

Prof. Dr. Gamal Abd-El-Aty.
Dr. Walaa Ibrahim Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr. Duaa Mohamed

Printing Date : 14/04/2026 10:00:20

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient's Name: Sabrya Mohammed Marai Farhan
Date: 2026-09-07
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Bilateral high resolution sonography revealed:

Clinical summary: Status post left MRM , routine follow up.

Findings:

Right breast.

- Normal breast parenchymal with mottling parenchymal appearance
- no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left mastectomy bed.

- Clear mastectomy bed with no masses or fluid collections.
- Clear left axilla. No pathological supra or infra-clavicular lymph nodes.
-

Conclusion:-

- *Right breast (BIRADS 2):NORMAL*
- *Clear left mastectomy bed and axilla.*

Recommendation:-

- *For annual follow up.*

Best Regards,
DR. Marwa M.Hassan



Patient's Name: Sabrya Mohammed Marai Farhan
Date: 2026-08-31
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Examination:

■ Computed tomography of the chest, abdomen and pelvis

Technical data:

- 5 mm CT scans of the chest, abdomen and pelvis with IV contrast

Clinical data: known case of operated left breast cancer

Comparison: in correlation to 12-03-2025 report

Findings:

Chest findings:

- Left mastectomy with operative bed scar irregular thickening
- Left upper lung lobe patchy ground glass densities
- No definite pulmonary nodules
- No hilar or mediastinal masses or sizable LNs.
- Normal appearance of the tracheobronchial tree.
- No evidence of pleural effusion or thickening.
- Mildly enlarged cardiac size, no pericardial effusion.
- No suspicious axillary LNs on CT base

Abdomen and pelvis findings:

- The liver shows enlarged size with cirrhotic features, no evidence of focal lesions.
- No intra hepatic biliary dilatation.
- Dilated CBD 9 mm with detected faint hyperdense focus at its distal part measures 10mm.
- Normal caliber and homogenous enhancement of the portal vein.
- The spleen shows normal size. No evidence of focal lesions.
- Normal CT appearance of the gall bladder, pancreas and adrenal gland
- Left renal upper calyceal stone 5mm/500HU, otherwise kidneys are of normal size, size and preserved parenchymal thickness, no other stones, back pressure, cysts or masses bilaterally.
- The urinary bladder shows average distention, no wall thickening, no stones, diverticulae or masses.
- No suspicious abdominal or pelvis enlarged LNs
- No CT evidence of intestinal obstruction



Patient's Name: Sabrya Mohammed Marai Farhan
Date: 2026-08-31
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- No pneumoperitoneum
- No free or localized abdominal or pelvis fluid collection
- Right acetabulum and ischium 3 cm lytic like lesion

Opinion:

- Left mastectomy with operative bed scar irregular thickening for dedicated breast studies assessment/ PET-CT
- Left upper lung lobe patchy ground glass densities likely infectious/ inflammatory
- No definite pulmonary nodules
- Enlarged cirrhotic liver
- Dilated CBD 9 mm with detected faint hyperdense focus at its distal part measures 10mm, possible stone, possible lesion for MRCP
- Right acetabulum and ischium 3 cm lytic like lesion

Recommendation:

- PET-CT / bone scan correlation
- MRCP

Regards, Dr. Ahmed Rezk, MD.

Ahmed

Ahmed

